While the decline in US overdose deaths represents a major step forward, the overdose crisis is far from over, and recent gains remain fragile. Sustaining progress will require nimble investments driven by the understanding of the factors that have driven progress to date and where the unexploited opportunities lie. Prematurely abandoning interventions that have demonstrated effectiveness risks reversing hard-won progress, particularly because public health successes are often most vulnerable once they begin to show results. Whether this historic decline proves to be a durable turning point or only a temporary reprieve likely depends on decisions made now.

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Indonesia's health reform: from pandemic mandate to the six pillars of transformation

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In Indonesia, a large, diverse, middle-income country, the COVID-19 pandemic placed unprecedented pressure on the health system, exposing health inequities and inefficiency, and underscoring the urgency for structural reform. When I was appointed as Minister of Health at the height of the COVID-19 pandemic in December, 2020, my mandate was two-fold: to manage the immediate crisis and to deliver the most ambitious health reform in Indonesia's history.

The pandemic laid bare the health system's vulnerabilities: wide geographical and socioeconomic disparities in service capacity, including capacities of health workers, specialists, and technologies, compounded by fragmented governance, financing, and data systems.[1] Although Indonesia has made substantial progress in demand-side health financing, achieving over 95% population coverage under our national health insurance programme, Jaminan Kesehatan Nasional,[2]

coverage alone is not enough. A universal health coverage card offers little protection if local services are under-resourced or the necessary specialist is hundreds of kilometres away. The reform objective was clear: to ensure that all 280 million Indonesians can access affordable, high-quality health care, regardless of where they live.

To turn this ambition into reality, the Government of Indonesia is rebuilding the health system's foundation. In 2023, a landmark Omnibus Health Law was enacted that consolidates 11 fragmented health laws into one and addresses long-standing regulatory bottlenecks.[3] The law has since been operationalised through key government, presidential, and ministerial regulations, providing the legal backbone for addressing long-standing supply-side constraints and accelerating the six pillars of health transformation.

First, the Government of Indonesia is aggressively shifting the balance of the health system towards primary care and prevention, reducing over-reliance on hospital-centric models of care that are increasingly difficult to finance sustainably. For an archipelagic country spanning more than 17 000 islands, this shift is essential to bring care closer to people. Indonesia is strengthening its nationwide primary health-care network, comprising more than 10 337 existing primary health centres and public health laboratories across 514 districts, by standardising services and shifting from a disease-based model to a life-course approach.[4] In February, 2025, the President of Indonesia launched an ambitious nationwide free health screening programme, Cek Kesehatan Gratis, to enable early detection of non-communicable diseases. Since the launch, more than 100 million people have been screened, with 16·8 million requiring follow-up care.[5]

Second, the Government of Indonesia is transforming secondary care by tackling gaps in access to lifesaving technologies nationwide. Indonesia has shifted from fragmented decentralised purchasing to centralised, long-horizon procurement that bundles equipment, installation, training, and maintenance, helping ensure that public investment translates into usable, well maintained services. This procurement reform allows us to deploy medical devices, including catheterisation laboratories, mammography equipment, and CT scanners, ensuring that the geographical lottery no longer dictates a patient's chances of survival from cardiovascular disease or cancer.[6]

Third, the Government of Indonesia is strengthening health system resilience. The COVID-19 pandemic exposed our reliance on global supply chains, with over 90% of active pharmaceutical ingredients and medical devices imported.[7] We are therefore expanding local manufacturing for medical countermeasures. Vaccine production capacity has grown from over 2 billion doses in 2018 to over 3·5 billion in 2026,[8] and manufacturers have increased from one to four.[9,10] For therapeutics, Indonesia established its first plasma fractionation facility in 2023, with local production of essential plasma-derived medicines expected by 2027.[11]


Juni Kriswanto/AFP via Getty Images

Fourth, the Government of Indonesia is working to improve health financing effectiveness. With a national health budget exceeding IDR 218·5 trillion in 2025 (about US$13 billion in 2025), the Government of Indonesia is strengthening the alignment between national and subnational health spending by implementing national health accounts, which systematically track health expenditure across funding sources and levels of government.[12] Additionally, the Ministry of Health is increasing the pooled procurement of medicines to improve efficiency. In 2025, Ministry of Health hospitals consolidated the procurement of 633 medicines and consumables, generating savings of IDR 806 billion, corresponding to a reduction of approximately 32%.[13]

Fifth, the Government of Indonesia is expanding and redistributing our health workforce. Indonesia faces a severe deficit of doctors: only 0·69 per 1000 population, with doctors heavily concentrated in Java island.[14] The specialist workforce gap is particularly acute: Indonesia had a shortage of more than 31 000 specialists in 2023, a deficit projected to exceed 65 000 by 2032—more than doubling within a decade.[15] In response, Indonesia has increased scholarship awards for specialist training in priority fields more than four-fold, from 600 in 2021 to more than 2500 from 2024 onwards.[16] But scholarships alone cannot close the gap without expanding the capacity to train specialists. The 2023 Omnibus Health Law therefore enabled hospital-based specialist programmes as an additional pathway alongside traditional university-based training, with six new programmes and 167 trainees in the first three cohorts since 2025.[17]

Sixth, the country is advancing health technology across digital technology and biotechnology. Leveraging our COVID-19 tracing platform, which reached over 104 million users,[18] the Ministry of

Health is rolling out SATUSEHAT—an interoperable national health data platform that integrates electronic medical records from over 146 000 target facilities.[19] In parallel, the Ministry of Health leveraged investments in COVID-19 genomic surveillance to launch the Biomedical and Genome Science Initiative, Indonesia's first national population-scale genomics initiative. To date, this initiative has enrolled over 20 000 participants and generated more than 16 000 human whole-genome sequences.[20]

Taken together, these reforms show how a mandate born in crisis has evolved into a long-term agenda for transforming the health system. The pandemic showed that resilience is built before a crisis: through services that are close to people, referral systems that are ready when needed, supply chains that withstand shocks, effective financing, a workforce that is distributed according to need, and connected data systems. Implementation remains the central test. In Indonesia's decentralised health system, national ambition must be translated into consistent delivery across diverse local contexts. This requires sustained alignment across government, stronger local delivery capacity, data-driven accountability, and political and financial commitment over time.

The health gaps in Indonesia will not be closed immediately, and some impacts of the health reforms will take time to materialise. But Indonesia has chosen a path that seeks to be fair, fiscally sustainable, and institutionally strong. With sustained domestic commitment and international partnership, Indonesia can turn this reform into lasting institutional change and build a health system the next generation can rely on.

Since 2020, I have been the Minister of Health of the Republic of Indonesia. I am a Board member of Gavi, the Vaccine Alliance; Co-Chair of the WHO TB Vaccine Accelerator Council; Co-Chair of the High Level Panel for Accra Reset; Chair of the Board of Trustees of Institut Teknologi Bandung and the Mandiri Amal Insani Foundation; and member of the Board of Trustees of Institut Teknologi Sepuluh Nopember. During the preparation of this Comment I used ChatGPT (GPT-5.5) to support language editing and grammar checking only and take full responsibility for the content of the Comment.

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